PACKAGE INSERT

BENIL TABLET

Each tablet contains:
Glibenclamide 5mg

Product Description: Oblong, flat, white, plain tablet with single score on one side only.

Pharmacodynamics:
Glibenclamide is sulphonyl urea oral hypoglucemic agent. Though the precise mechanism of action is not completely understood, the mode of action of glibenclamide is believed to be that of stimulation of synthesis and release of endogenous insulin. Glibenclamide has similar actions and uses to chlorpropamide. Glibenclamide is believed to be able to stimulate the islet tissue in the pancreas to synthesize and to secrete insulin. It is generally used to control the mild to moderately severe maturity onset, stable diabetic. Although glibenclamide is a sulphonamide derivative, it is devoid of anti-bacterial activity.

Pharmacokinetics:
Glibenclamide is readily absorbed from the gastro-intestinal tract and is extensively bound to plasma proteins. It is excreted in the faeces and as metabolites, in the urine. The metabolism of glibenclamide was studied in 6 healthy adult volunteers. After administration of 5mg by mouth, about 45% of the dose was absorbed, peak concentrations in blood were reached 2 to 4 hours after administration, and the concentration of glibenclamide was twice as high in plasma as in whole blood. The half-life in plasma was about 5 hours. Two hydroxylated metabolites had been identified and a third unidentified metabolite was present in small amounts. The metabolites had no significant hypoglycaemic activity and were excreted in the bile and urine. Of the total excreted in the urine (about 24%) within 3 days, about one half was excreted within 6 hours and almost all of the remainder within 24 hours; of 72% excreted in the faeces within 5 days, about 37% was excreted within 24 hours.

Indication:
Maturity onset diabetes mellitus (non-insulin dependent diabetes mellitus) which cannot be completely controlled by diet, exercise and weight reduction alone. It can often be used in patients of this type in place of insulin.

Recommended Dosage:
The dosage of glibenclamide is governed by the desired blood sugar level. The dosage of glibenclamide must be the lowest which is effective. The treatment with glibenclamide must be initiated and monitored by a doctor. The patient must take glibenclamide at the times and in the dose prescribed by the doctor. Mistakes, e.g. forgetting to take a dose, must never be corrected by subsequently taking a larger dose. Measures for dealing with such mistakes (in particular forgetting a dose or skipping a meal or in the event a dose cannot be taken at the prescribed time) must be discussed and agreed between doctor and patient beforehand. If it is discovered that too high a dose or an extra dose of glibenclamide has been taken, a doctor must be notified immediately.
The usual initial dose in non-insulin-dependent diabetes mellitus is 2.5 to 5 mg daily by mouth, with breakfast, adjusted every 7 days by increments of 2.5mg daily up to 15mg daily. Although increasing the dose above 15mg is unlikely to produce further benefit, doses of up to 20mg daily have been given. Doses greater than 10mg daily may be given in 2 divided doses. Debilitated, malnourished or elderly patients, or those with impaired renal or hepatic function: A daily dose of 2.5mg is recommended initially. Patients should have their blood or urine glucose monitored every 3 to 5 days and if a dosage increase is deemed necessary increments of not more than 2.5 mg daily should be prescribed at weekly intervals.
<u>Initial Dosage in Patients Transferred from Other Antidiabetic Agents:</u>
Sulphonylureas: the switch to glibenclamide can be immediate. An initial dose of 10mg should not be exceeded. Subsequent dosage adjustments are based on the patient's blood or urine glucose response. If patients were on chlorpropamide they should be closely monitored for hypoglycaemia for the first 2 weeks of the transition.Biguanides: 2.5mg glibenclamide can be substituted for the biguanide initially and adjusted as required after 3 to 5 days. On occasion, glibenclamide may be prescribed together with metformin if control is not adequate with either agent alone. Clinical benefit of the combination should be monitored from time to time.

Route of Administration: Oral

Contraindications:
Use of glibenclamide is contraindicated in:
a) insulin-dependent diabetes.
b) diabetes complicated by ketosis and acidosis, major surgery, severe trauma or infections.
c) coma and precoma diabetes.
d) severe renal and hepatic dysfunction.
e) hypersensitivity to glibenclamide or to any of the excipients
f) pregnancy and lactation.

Warnings and Precautions:
Alertness and reactions may be impaired by hypoglycaemic or hyperglycaemic episodes, especially when beginning or after altering treatment or when glibenclamide is not taken regularly. This may for example, effect the ability to drive a vehicle or cross the road safely, or to operate machinery.To achieve the goal of treatment with glibenclamide-optimal control of blood sugar-adherence to correct diet, regular and sufficient physical exercise and if necessary, reduction of body weight are just as necessary as regular intake of glibenclamide.The signs of hyperglycaemia and hypoglycaemia are:
<u>Hyperglycaemia:</u>
Severe thirst, dryness of mouth, frequent micturition, dry skin.
<u>Hypoglycaemia:</u>
Intense hunger, sweating, pallor, tremor, restlessness, irritability, depression of mood, headaches, disturbed sleep and impairment of performance and alertness.Hypoglycaemia may result from overdosage of glibenclamide, from interactions with certain drugs or from reduced dietary intake. Hypoglycaemia will resolve within minutes of taking glucose by mouth; either sugar tablets, some sweet food (e.g. sweet biscuits), or a sweet drink with added sugar. It is wise to check the blood glucose when recovered to ensure that the blood glucose level has risen. If hypoglycaemia persists, or if a missed or delayed meal has brought the hypoglycaemia on, then it is necessary to have a meal in order to correct the blood glucose level and prevent a further relapse.In exceptional stress situations (e.g. trauma, surgery, febrile infections), blood glucose regulation may deteriorate, and a temporary change to insulin may be necessary to maintain good metabolic control. Persons allergic to sulphonamide derivatives may develop an allergic reaction to glibenclamide as well. Glibenclamide should be avoided in elderly patients and debilitated and malnourished patients as they are particularly susceptible to the hypoglycaemic effects of sulphonylureas.

Interactions with Other Medicaments:
Patients who take or discontinue taking certain other medications while undergoing treatment with glibenclamide may experience changes in the blood sugar control.
<u>Attenuation of blood sugar lowering effect:</u>
Salicylates, phenylbutazone, sulphonamides, chloramphenicol, coumarin anticoagulants, clofibrate, monoamine oxidase inhibitors, fenfluramine, sulfinpyrazone, tetracyclines, insulin and other oral antidiabetic, ACE inhibitors, anabolic steroids and male sex hormones, azaproprazone, cyclophosphamide, disopyramide, fenyramidol, fibrates, fluoxetine, ifosfamide, miconazole, oxyphenbutazone, paraaminosalicylic acid, pentoxifylline (high dose parenteral), probenecid, quinolones, sympatholytic agents (such as beta blockers and guanethidine), tritoqualine and trofosfamide.
<u>Weakening of the blood sugar lowering effect:</u>
Acetazolamide, barbiturate, other sympathomimetic agents, glucagon, nicotinic acid, progesterone, phenothiazines, thyroid hormones, rifampin, corticosteroids, oral contraceptives, thiazide diuretics, chlorpromazine, diazoxide, estrogens, phenytoin and abuse of laxatives. Concomitant treatment with sympatholytic drugs, clonidine or reserpine may mask the warning symptoms of a hypoglycaemic attack. Potentiation or attenuation of the blood sugar lowering effect of glibenclamide has been observed during concomitant therapy with H_2-receptor antagonists. Intolerance to alcohol may occur. Excessive alcohol ingestion by people who drink occasionally may attenuate the hypoglycaemic effect of glibenclamide or dangerously potentiate it by delaying its metabolic inactivation. Disulfiram-like reactions have occurred very rarely following the concomitant use of alcohol and glibenclamide.

Pregnancy and Lactation:
Insulin is preferred for the treatment of pregnant patients. Safe conditions for the use of glibenclamide in pregnancy have not been established, and serious consideration should be given to the potential hazard of its use in women of childbearing age who may become pregnant.

Side Effects:
Skin sensitization, gastro-intestinal disturbances, leucopenia, and jaundice may occur. In the event of sore throat or fever, repeated white-cell counts should be carried out because abnormalities in the circulation blood may not appear for several days. Hypoglycaemia occurs with hypoglycaemic sulphonylureas. The incidence of hypoglycaemia can be reduced if glibenclamide is taken with or immediately after a meal. Side-effects include skin rashes, photosensitivity, diarrhoea, nausea, vomiting, epigastric pain, feeling of gastric fullness, dizziness, headache, weakness, fever, hypoglycaemic reactions and paraesthesia. Cholestasis, hepatitis, jaundice, blood disorders including eosinophilia, leucopenia, thrombocytopenia, aplastic anaemia, pancytopenia, haemolytic anaemia and agranulocytosis may occur. Transient visual disturbances may occur at the commencement of treatment.

Symptoms and Treatment of Overdose:
Hypoglycaemia occurs as a result of overdosage. The early symptoms are weakness, giddiness, pallor, sweating, hunger, palpitations, irritability, nervousness, headache and tremor. Later symptoms are depression or euphoria, inability to concentrate, drowsiness, lack of judgement and self-control and amnesia. Other symptoms are hemiplegia, abnormal reflexes, ataxia, tachycardia, diplopia, paraesthesia delirium and unconsciousness. Finally leading to convulsion and coma.Treatment of overdosage is mainly to control the development of hypoglycaemia. In the conscious and cooperative patient, hypoglycaemia should be treated by the oral administration of a readily-absorbable form of carbohydrate, such as sugar lumps or a glucose-based drink. If coma occurs, up to 50ml of 50% solution of dextrose should be given intravenously, or dextrose or sucrose may be given by stomach tube. If dextrose is not immediately available, glucagon 0.5 to 1mg by subcutaneous or intramuscular or intravenous injection may be given. One or two further doses may be given at 20 minutes intervals if required. Following a return to consciousness, carbohydrate by mouth may need to be given until the effect of the drug has ceased. The patient should be observed over 3 to 5 days in case hypoglycaemia recurs.

Storage Conditions: Store at or below 30°C. Protect from light.

Pack Size: Blister: 10 x 10, 50 x 10 and 100 x 10 tablets.
Pack Size (export only): Bottle: 100 and 1000 tablets.

Shelf-life: 3 years.

FURTHER INFORMATION CONCERNING THIS DRUG CAN BE OBTAINED FROM YOUR FAMILY PHYSICIAN / LOCAL GENERAL PRACTITIONER / PHARMACIST.

Manufacturer & Product Registration Holder:
SUNWARD PHARMACEUTICAL SDN. BHD.
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